

PERSONALIZED HEALTH ANALYSIS

Marco Maciel

2026-08-09 · Consultant: Johnny / Irene · Recheck in 4 Weeks

Fatty Liver + Metabolic Burden

Constipation — Hard Pellet Stool

Anxiety + Motion Sickness + Circulation Cluster

CLIENT OVERVIEW

NAME

Marco Maciel

AGE

29

GENDER

Male

REFERRED BY

(to confirm)

BODY COMPOSITION

WEIGHT

220 lb

Above ideal for height

HEIGHT

175 cm

5'9"

BMI (CALC.)

32.6

Class I obesity (30–34.9)

SMART SCALE

Not captured

Priority for next recheck

Smart scale was not captured this session — this is the first priority at the 4-week recheck to get visceral fat, body fat %, skeletal muscle, and metabolic age baselines. BMI of 32.6 places Marco in Class I obesity. At 29 with a self-reported fatty liver and hard-pellet stool, his liver and gut are under meaningful daily stress. The positive picture: good water intake (1.5–2 L), good sleep timing (before 10 PM), and a protein-at-breakfast habit — these are real assets to build from.

TOP 3 PRIORITY SIGNALS

1 Fatty Liver + Metabolic Burden

Fatty liver marked Often at 29, combined with BMI 32.6 and frequent binge eating, is the most medically significant signal in Marco's profile. Non-alcoholic fatty liver disease (NAFLD) at this age is a direct consequence of excess caloric intake being stored as liver fat — it is common, serious, and highly reversible with dietary changes and targeted omega-3 support.

2 Constipation — Hard Pellet Stool

Hard pellet stool (Bristol Type 1) despite adequate water intake suggests a fiber deficit rather than dehydration. The liver and gut are directly connected — constipation slows the clearance of bile acids and metabolic waste that the liver processes, increasing the liver's daily load. Resolving constipation is both a quality-of-life improvement and a liver support intervention.

3 Anxiety + Motion Sickness + Circulation Cluster

Anxiety marked Often, motion sickness Often, occasional dizziness and headaches, and occasional blurry vision and light sensitivity form a pattern that may reflect a combination of autonomic nervous system sensitivity and reduced cerebral circulation — both of which are associated with obesity and omega-3 deficit. This is not a diagnosis, but it is a pattern worth monitoring.

ROOT CAUSE ANALYSIS

● Fatty Liver / Hepatic Load (Primary Driver)

Frequent self-reported fatty liver at 29 reflects the liver's ongoing struggle to process the metabolic load from excess caloric intake. The liver converts excess calories — particularly from refined carbohydrates and dietary fat — into triglycerides for storage, and when this exceeds its clearance capacity, fat accumulates in liver cells. The binge eating pattern (marked Often) means Marco is regularly presenting the liver with caloric surges beyond what it can immediately process. Omega-3 EPA has direct clinical evidence for reducing liver triglyceride accumulation by downregulating the enzyme pathways that produce liver fat. Soluble fiber supports liver detoxification by binding bile acids in the gut and preventing their reabsorption, reducing the liver's recycling load.

● Constipation / Gut (Secondary Driver)

Hard pellet stool despite adequate water is a fiber story, not a hydration story. Marco drinks 1.5–2 L of water, which is reasonable, but without adequate fiber the water passes through without forming a stool-bulking gel. Bristol Type 1 (hard pellets) indicates transit time is too slow — waste is sitting in the colon longer than it should, allowing reabsorption of metabolic byproducts the liver is trying to clear. Binge eating occasionally suggests hunger and satiety signals are dysregulated — soluble fiber directly addresses this by slowing gastric emptying and reducing appetite spikes. The gut and liver are in a bidirectional relationship: constipation worsens liver load, and liver dysfunction reduces bile output needed for normal digestion.

● Anxiety / Autonomic Sensitivity

Anxiety marked Often at 29 with frequent motion sickness (vestibular sensitivity), occasional dizziness, and headaches suggests some autonomic nervous system hyperreactivity. Omega-3 DHA is the dominant fatty acid in the brain and has the strongest evidence base for reducing anxiety and supporting vestibular function. At BMI 32.6 with NAFLD, the inflammatory load itself raises cortisol and sensitizes the stress response — reducing inflammation through diet and omega-3 supplementation is a direct anxiety intervention, not just a metabolic one.

SUMMARY

Marco's profile has a clear connection: fatty liver, constipation, and binge eating are all feeding each other through the gut-liver axis. The liver is stressed, the gut is slow, and hunger signals are dysregulated — producing a cycle of overconsumption and under-clearance. The interventions here are targeted: fiber to clear the gut and buffer blood glucose, omega-3 to reduce

liver fat and calm the nervous system, and movement to improve metabolic clearance. His good water intake and sleep timing are genuine assets.

30-DAY PLAN

1 Fiber at Every Meal

- Add one fiber source to every meal: vegetables, legumes (beans, lentils), or a fiber supplement.
- His stool (hard pellets) will normalize within 5–7 days as fiber intake increases — this is one of the fastest-responding changes.
- Aim for 25–30 g fiber/day. A serving of beans at lunch + vegetables at dinner = approximately 12–15 g.
- High-fiber breakfast: oats, fruit, or a high-fiber protein smoothie add another 5–8 g.

2 Reduce Binge Eating — Structured Meals

- Eat at consistent times: breakfast, lunch, dinner at set times reduces the blood sugar swings that trigger binge eating.
- Protein + fat + fiber at every meal slows gastric emptying and extends satiety — the combination matters.
- He already eats breakfast with protein (good). Add a fiber source to breakfast to further extend satiety.
- If binge urges hit, drink 500 ml water first and wait 15 minutes — often it is thirst or blood sugar, not true hunger.

3 Movement — Reduce Liver and Visceral Fat

- 20–30 minute walk daily — exercise is the single most effective intervention for NAFLD independent of diet.
- Break up prolonged sitting: stand and walk for 5 minutes every hour. This alone improves post-meal glucose clearance.
- Resistance training 2x per week (even bodyweight exercises) builds skeletal muscle that acts as a glucose sink.

4 Reduce Processed Food — Support Liver Recovery

- Processed food (fast food, packaged snacks, fried items) are the primary dietary source driving NAFLD progression.
- Replace one processed meal per day with a whole-food option — don't aim for perfection, aim for reduction.

→ The liver recovers fastest when the daily caloric and inflammatory load is consistently reduced.

SUPPORT TOOLS (OPTIONAL)

Food, water, sleep, and movement are always the foundation. These are targeted bridge tools for specific gaps.

Advanced Omega

EPA in Advanced Omega has the strongest nutritional evidence base for reducing liver fat in NAFLD — it directly inhibits the enzyme (DGAT-2) responsible for converting excess calories into liver triglycerides. DHA supports the nervous system and may reduce Marco's motion sickness and anxiety symptoms, as it is the dominant fatty acid in vestibular and brain tissue. At BMI 32.6 with a self-reported fatty liver, this is one of the highest-leverage targeted interventions available.

Take 2 softgels with the largest meal of the day. Liver fat and motion sickness effects build over 6–8 weeks — set this expectation. Anxiety improvements may be noticeable earlier.

Prebiotic Fiber

Marco's hard-pellet stool and binge eating pattern are both fiber-deficit problems. Prebiotic fiber provides the soluble fiber that bulks stool (resolving the constipation quickly), slows gastric emptying to extend satiety and reduce binge urges, and binds bile acids in the gut — reducing the amount the liver needs to reprocess daily. The gut-liver axis means improving gut motility directly reduces liver workload.

Mix one serving in a large glass of water before dinner or the largest meal of the day. Stool consistency change is typically noticeable within 5–7 days.

WHAT TO EXPECT IN 30 DAYS

- ✓ Improved stool consistency (less hard, more regular) within 5–7 days as fiber increases
- ✓ Reduced binge eating frequency within 1–2 weeks as fiber and structured meals stabilize satiety
- ✓ Less anxiety and motion sickness sensitivity at 4–6 weeks as omega-3 DHA builds in nervous tissue
- ✓ Improved post-meal comfort and energy as liver and gut function improve
- ✓ Full smart scale baseline captured at 4-week recheck: visceral fat, body fat, metabolic age
- ✓ Weight trending down at 4-week recheck with consistent fiber, movement, and reduced binge eating

NEXT STEP

OPTION A — LIFESTYLE ONLY

Fiber source at every meal, 20-min walk daily, structured meal times, reduce one processed meal/day. Recheck at 4 weeks: weight, stool consistency, binge eating frequency.

OPTION B — LIFESTYLE + SUPPORT

Advanced Omega with largest meal + Prebiotic Fiber before dinner, plus all lifestyle changes above. Recheck at 4 weeks: weight, full smart scale baseline, stool form, binge frequency, anxiety level (1–5).

4 WEEKS RECHECK

Weight, full smart scale (first reading), stool consistency (Bristol scale), binge eating episodes/week, anxiety level (1–5), motion sickness frequency

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